Organizing the intersectoral response to complex and co-occurring social needs in primary care

Configuring networks adapted to the different profiles of patients in care



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01 Context

In primary care, social needs pile up

54 %
of patients have at least one unmet social need
Jih et al., 2023

351
different combinations of two or more needs
Kreuter et al., 2021

THREE DISTINCT NOTIONS

Co-occurrence is counted; complexity is described; the intersectoral response is organized within a territory.



INCLUSION CRITERION
Co-occurrence
At least two social needs present at the same time.
Kreuter et al., 2021



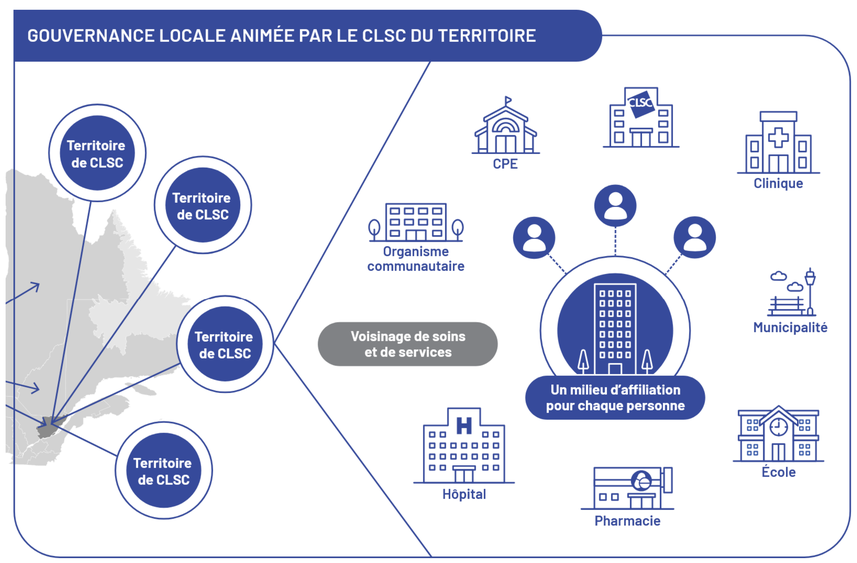
DESCRIBED, NOT REQUIRED
Complexity
Accumulation of chronic diseases, disabilities, mental health problems and social vulnerability.
Kuluski et al., 2017



ORGANIZED WITHIN A TERRITORY
Intersectoral response
Actions by sectors other than health, carried out with it, on health or its determinants: CLSC, hospital, family medicine groups, pharmacies, community organizations, municipality, schools, childcare, private actors.
MSSS, 2017; Leppo et al., 2013

QUÉBEC PRIMARY CARE POLICY · MSSS, 2026

The CLSC territory becomes the reference unit



FOUR ROLES OF THE CLSC

Navigator · Coordinator · Intersectoral leader · Co-facilitator

"in a spirit of shared governance" · MSSS, 2026, p. 42 · figure: MSSS, 2026 (in French)

MP FICTIONAL CASE · COMMON THREAD

Ms. Protocol · 58

Who responds? With whom? Who coordinates?

Eviction notice → Food insecurity → Debts → Depression (clinical context)

? THE GAP

The CLSC must coordinate. Yet for a given combination of needs, nothing says which partners to mobilize, how to link them, or who coordinates.

RESEARCH QUESTION

How is the intersectoral response organized for the different profiles of patients with complex and co-occurring social needs cared for in primary care?

Aim: link each profile to...

A needs profile → a configuration of partners → and its coordinator

02 Methods

Setting: one CLSC territory in Lanaudière, Québec · multiphase design · O2: CLSC ego network, explanatory sequential mixed methods

Which profiles?

O1 · IDENTIFY
Scoping review

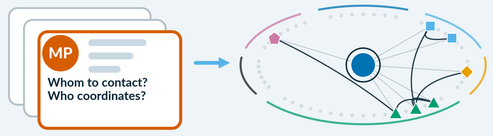
Published studies → Recurring profiles → Vignettes

→ 4–6 validated vignettes

MP *Her profile is found in the literature*

Whom to mobilize?

O2 · ANALYZE
CLSC ego network



→ one configuration per profile

MP *She becomes a vignette*

Why these choices?

O2 · ANALYZE
Map-based interviews



→ explanations per profile

MP *Her map guides the interviews*

What to recommend?

O3 · RECOMMEND
Deliberative workshop



→ recommendations per profile

MP *Her case is deliberated in the workshop*

Integrate

2026 → 2029
Joint display by profile

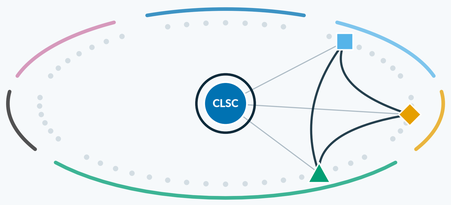
Profile	Network	Why	Recs
A			
B			
C			

MP *Guidance for her profile*

03 Expected results

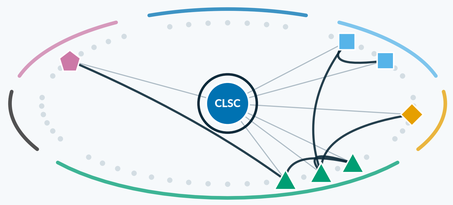
Fictional examples · **the CLSC ego network changes with the profile**

A Housing-dominant
eviction notice



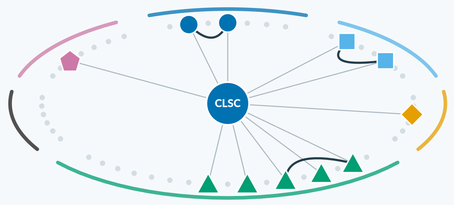
3 partners, all linked · the CLSC coordinates

B Ms. Protocol
housing · food · debts



7 partners, still linked · the CLSC coordinates

C Accumulation + depression
same needs, plus depression



11 partners, loosely linked · no coordinator

CLSC (ego) · designated coordinator · tie between partners · organization not mobilized
Health and social services · Primary care, pharmacies · Community · Municipal · Education, early childhood · Employment, justice, private

H1 Sectors change with the profile
H2 A common core, a specific periphery
H3 More partners, but less connected
H4 Does the CLSC navigate and coordinate?
H5 Does gender change the response?

04 Impacts and outcomes

Not a universal "ideal network": guidance for each profile.

For people
A response tailored to their combination of needs

For the CLSC and its partners
Guidance: whom to mobilize, how to connect, who coordinates

For primary care policy
A baseline; recommendations aligned with the CLSC's four roles

For research
A profile–network hypothesis; one ego network per profile, built from vignettes

French, colour-blind and audio versions
Français · daltonisme · audio

Comment, suggest, get in touch
Readings · contacts · methodological ideas

References: Creswell & Plano Clark, 2017 · Gale et al., 2013 · Jih et al., 2023, BMC Primary Care · Kreuter et al., 2021, Population Health Management · Kuluski et al., 2017, Int J Integr Care · Leppo et al., 2013 · Leutz, 1999 · MSSS, 2017; 2026, Québec primary care policy · Peters et al., 2020 · Thorne, 2016 · Colour-blind version: Okabe-Ito colours and one shape per sector